INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Vismodegib for advanced basal cell skin cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00236
Version 3

Reference calendar for daily medicine and reviews

Treatment stages

01 **Continuous oral treatment**
Daily medicines

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call urgently for blistering or peeling skin, mouth sores with rash or fever, severe muscle pain or weakness, or dark urine. Seek immediate advice about suspected pregnancy or exposure. Report taste change, reduced eating and weight loss early so support can start.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

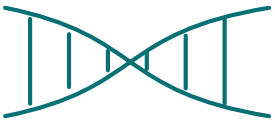
Medicines in this regimen

Vismodegib is used to help control advanced basal cell skin cancer when the source-specific criteria are met. Discuss whether the aim is shrinkage, symptom relief, longer disease control or another agreed goal, and how surgery or radiotherapy has been considered.

ORAL TARGETED CAPSULE

Vismodegib

Erivedge



Blocks the Hedgehog growth pathway

Vismodegib blocks Smoothened, a protein in the Hedgehog signalling pathway. Abnormal activity in this pathway can drive basal cell cancer. Blocking it can slow tumour growth, while effects on normal tissues help explain cramps, taste changes and hair loss.

Daily capsule → Hedgehog signal blocked → Cancer growth may slow

What to watch for

Keep four-week blood tests and the pregnancy-prevention programme. Report cramps, loss of strength, reduced intake, weight loss and severe skin symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis and treatment goal

Use these questions with your own pathology report and agreed treatment plan.

Confirm the diagnosis and extent

Basal cell carcinoma starts in the skin. It usually grows locally; spread to distant organs is uncommon. Your team assesses the affected area and whether surgery or radiotherapy can treat it.

Why tablets are being considered

Vismodegib is used in the advanced basal cell carcinoma settings specified in your treatment protocol. Ask the team to explain why this medicine is suitable and why an operation or radiotherapy alone is not the agreed option.

Record earlier local treatment

Record previous operations, radiotherapy and other treatment to the same area. These details help the team plan further treatment and interpret changes in the skin.

Inherited conditions and multiple cancers

Several basal cell cancers at a young age or a suggestive family history may warrant a genetics assessment. Gorlin syndrome is one possible inherited cause. Testing is selected for the person; it is not required for every skin cancer.

A visible lesion is only one part of assessment

The team may measure or photograph a visible lesion and examine deeper tissue or lymph nodes. Scans are used when they answer a specific question. A photograph alone cannot show the full extent of disease.

Your medicines and the overall plan

The schedule in this setting

Take vismodegib once every day at the same time, continuously while the team considers it beneficial and tolerable. A 28-day calendar organises the four-week review; it is not a planned seven-day break or a fixed number of courses.

The role of earlier or later treatment

Ask how the medicine fits with previous or planned local treatment. The timing of an operation, radiotherapy and a tablet course should be written in one agreed plan.

Pauses and stopping

A safety pause is an active treatment decision. Record which medicine is paused, the reason, the tests or recovery needed and who may authorise restarting. Do not restart from an old prescription or reset the history when a new phase begins.

Questions, decisions and practical support

Your priorities and the burden of treatment belong in the discussion.

Benefits and uncertainties

Ask what outcome this treatment aims to improve for your diagnosis and what alternatives exist. Results from a study group help a discussion but cannot promise your own result.

Alternatives and review points

Agree the next decision point and what would make the plan change. Ask about other suitable treatments, trials and symptom care. Support for comfort and function can continue alongside cancer treatment.

Help with daily life

Tell the team what is hardest in daily life: travel, work, caring duties, costs, sleep or confidence. Ask who can help and how to contact them.

Shared understanding

Before going home, check that you can describe your medicine schedule, next tests and urgent-contact route. Ask for an interpreter, larger print or a supporter if that would help.

Evidence in advanced basal cell carcinoma

ERIVANCE studied vismodegib without a comparison treatment. At an analysis 21 months after the last person joined the study, independent reviewers found a confirmed response in 11 of 33 people with metastatic basal cell carcinoma (BCC) (33.3%; 95% CI 19.2-51.8%) and 30 of 63 with locally advanced BCC (47.6%; 95% CI 35.5-60.6%). The analysis date does not mean everyone took treatment for 21 months.

A response means that the study's cancer-response criteria were met. The assessment combined scans for metastatic disease and lesion measurements, ulcer healing and biopsy information for locally advanced disease. Response is not the same as cure and does not prove longer survival compared with another treatment.

CHAPTER 02

Your schedule, tests and appointments

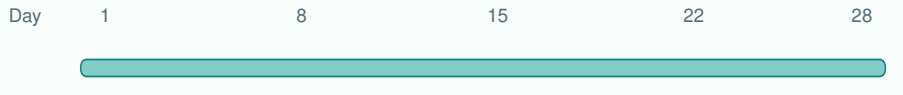
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

The overall treatment plan

Vismodegib for advanced basal cell skin cancer

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAILY MEDICINES

Continuous oral treatment

No preset cycle total | **Vismodegib daily; four-week safety review. No routine treatment-free week.**

Take vismodegib once every day at the same time, continuously while the team considers it beneficial and tolerable. A 28-day calendar organises the four-week review; it is not a planned seven-day break or a fixed number of courses.

The treatment setting determines duration; a reference calendar is not a prescription or a reason to restart a held medicine.

BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00236 v3: current regimen source · EMA Erivedge product information, updated 5 November 2025

THE WHOLE TREATMENT JOURNEY

Vismodegib for advanced basal cell skin cancer: treatment and checks

NCCP 00236: symptomatic metastatic basal cell carcinoma, or locally advanced basal cell carcinoma unsuitable for surgery or radiotherapy.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00236	Full blood count, kidney and liver tests; medicine review and pregnancy-prevention programme assessment for women and men. If pregnancy is possible, a supervised pregnancy test is needed within seven days before starting.	Full blood count, kidney and liver tests every four weeks. Review cramps, strength, taste, eating, weight, hair/ skin changes and treatment tolerance. Pregnancy testing is monthly when applicable, even if periods stop.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Agree continuing clinical/skin review, any response imaging and support for ongoing effects. The pregnancy-prevention, donation and breastfeeding restrictions continue after the last dose for their specified periods.
Cancer, skin and daily function NCCP 00236	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up covers the treated area and the rest of your skin. The team chooses examinations and scans for your situation. Vismodegib pregnancy-prevention and donation restrictions continue for the periods listed in your safety plan after the final dose.

Medicines and continuing precautions NCCP 00236	Review all medicines and the pregnancy/ fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Vismodegib can cause severe harm to an unborn baby. Women who could become pregnant need the formal pregnancy-prevention programme, including two recommended contraceptive methods (one highly effective method plus a barrier method), unless the agreed alternative is continuous abstinence, during treatment and for 24 months after stopping. Pregnancy tests are supervised before treatment and monthly, even with absent periods. Men must use a condom, with spermicide if available, during sex with a female partner during treatment and for two months afterwards, including after a vasectomy. Report a suspected patient or partner pregnancy immediately. Do not breastfeed or

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
					donate blood during treatment or for 24 months afterwards; do not donate semen during treatment or for two months afterwards. Ask the team to explain and document the full programme; this summary does not replace its materials.

What each check is for

1 Continuous oral treatment

Daily medicines

Vismodegib is used to help control advanced basal cell skin cancer when the source-specific criteria are met. Discuss whether the aim is shrinkage, symptom relief, longer disease control or another agreed goal, and how surgery or radiotherapy has been considered.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

Swallow the prescribed capsule whole with about 200 mL of water, with or without food, at the same time each day. Do not open, crush or chew it. Keep it in its labelled pack, away from children, and return unused capsules to the pharmacy.

STEP 2

Missed, vomited or held treatment

If a dose is missed, skip it and take the next dose at the usual time. Do not double up. If you vomit after taking it, ask the oncology team or pharmacist before taking more; do not assume that the dose needs replacing.

STEP 3

Appointments and supply

Plan blood tests, travel and medicine collection in advance. If you need an interpreter or someone to help at home, tell the team before the appointment.

STEP 4

Paper copies and contact information

Ask for your latest schedule and contact details on paper. Replace old versions when treatment changes so everyone helping you uses the same plan.

Taking treatment and preparing for visits

Keep the written medicine plan beside this guide; it records your own dose and dates.

Taking or receiving the medicine

Other medicines and supplements

Show the pharmacist a current list of prescriptions, over-the-counter medicines, vitamins, herbal products and supplements. Include anticoagulants, heart medicines, steroids and recent antibiotics. Check before adding, stopping or changing anything; an apparently harmless product can alter treatment or confuse a symptom assessment.

Practical preparation

Bring your medicine list, allergy information and questions. Tell the team about practical problems with travel, swallowing, wound care or remembering doses before treatment starts.

Follow-up and your continuing record

Keep this record when treatment is paused, changed or completed.

Your follow-up plan

Follow-up covers the treated area and the rest of your skin. The team chooses examinations and scans for your situation. Vismodegib pregnancy-prevention and donation restrictions continue for the periods listed in your safety plan after the final dose.

Continuing medicine precautions

Agree continuing clinical/skin review, any response imaging and support for ongoing effects. The pregnancy-prevention, donation and breastfeeding restrictions continue after the last dose for their specified periods.

Treatment summary for other clinicians

Record the medicines and last dates, important reactions, any long-term hormone replacement or steroid taper, and the responsible oncology contact. Show this information in urgent care and to other specialists. A different treatment phase does not erase earlier toxicity.

The complete paper version

Use this handbook as your paper record. Keep the current schedule, medicines, tests and contacts together, and replace outdated pages after a change. The matching QR link is optional.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Daily oral treatment	 **Days 1–28** Vismodegib daily; four-week safety review. No routine treatment-free week.
Safety reviews as scheduled	 **Days 1–28** Keep the specific blood, organ and skin-review appointments; they do not all follow the same interval.
Symptoms and support	 **Days 1–28** Call urgently for blistering or peeling skin, mouth sores with rash or fever, severe muscle pain or weakness, or dark urine. Seek immediate advice about suspected pregnancy or exposure. Report taste change, reduced eating and weight loss early so support can start.

Timing note 1: Take vismodegib once every day at the same time, continuously while the team considers it beneficial and tolerable. A 28-day calendar organises the four-week review; it is not a planned seven-day break or a fixed number of courses.

Timing note 2: If a dose is missed, skip it and take the next dose at the usual time. Do not double up. If you vomit after taking it, ask the oncology team or pharmacist before taking more; do not assume that the dose needs replacing.

Timing note 3: The formal pregnancy-prevention programme is essential; restrictions continue after stopping.

Timing note 4: A four-week review calendar does not create a routine drug holiday.

Timing note 5: Routine creatine phosphokinase (CPK) testing is not specified by the cited NCCP schedule; muscle symptoms need individual assessment.

Why these bands stay broad: This calendar shows the pattern. Your team supplies actual dates, prescribed doses and any hold or restart instructions. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00236 v3: current regimen source ↗ · EMA Erivedge product information, updated 5 November 2025 ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Between-treatment notes

Day 1–28: Every day in the reference calendar

Vismodegib daily; four-week safety review. No routine treatment-free week. Use the written medicine plan; report new symptoms and keep tests even during a treatment hold.

Support around the medicines

1 Formal pregnancy-prevention programme

The programme materials, supervised tests, contraceptive methods and after-treatment restrictions must be agreed and documented before the first dose; they apply even during a treatment pause.

2 An individual symptom plan

Ask which anti-sickness or other symptom medicines are prescribed, when to take them and when to call if they do not help. Do not start a borrowed medicine or alter treatment to manage a side effect yourself.

3 Your urgent contacts

Keep your actual oncology, out-of-hours and pharmacy contacts on the wallet card. Example numbers on a template must be replaced before you use it.

4 Pregnancy and fertility precautions

Vismodegib can cause severe harm to an unborn baby. Women who could become pregnant need the formal pregnancy-prevention programme, including two recommended contraceptive methods (one highly effective method plus a barrier method), unless the agreed alternative is continuous abstinence, during treatment and for 24 months after stopping. Pregnancy tests are supervised before treatment and monthly, even with absent periods. Men must use a condom, with spermicide if available, during sex with a female partner during treatment and for two months afterwards, including after a vasectomy. Report a suspected patient or partner pregnancy immediately. Do not breastfeed or donate blood during treatment or for 24 months afterwards; do not donate semen during treatment or for two months afterwards. Ask the team to explain and document the full programme; this summary does not replace its materials.

5 Eating, activity and practical help

Let the team know if cramps, altered taste or poor appetite make eating or moving difficult. Ask for practical food and activity advice that fits your symptoms and any other illness.

Tests: what is required and what each result means

Safety tests, tumour tests and response scans answer different questions.

Before treatment

Full blood count, kidney and liver tests; medicine review and pregnancy-prevention programme assessment for women and men. If pregnancy is possible, a supervised pregnancy test is needed within seven days before starting.

During treatment

Full blood count, kidney and liver tests every four weeks. Review cramps, strength, taste, eating, weight, hair/skin changes and treatment tolerance. Pregnancy testing is monthly when applicable, even if periods stop.

Scans and tumour markers

Ask the team to write the next assessment and who will arrange it. Bloods used to monitor kidneys, liver, hormones or muscle are not automatically measures of cancer response. There is no universal tumour-marker blood schedule for these skin-cancer regimens; a scan or extra blood test is requested for a specific clinical reason.

Getting the result

Keep a named contact for bookings and results. If an expected appointment or result is missing, call that service; do not take silence as confirmation that everything is complete.

What to expect between visits

You can feel well and still need the planned checks. New symptoms deserve attention even on a day without an appointment.

Your usual pattern

Record the start and change in symptoms, temperature if advised, bowel pattern, drinking, urine and the effect on normal tasks. A simple few-line record is enough. Use it to describe a change rather than trying to grade or diagnose a toxicity yourself.

Eating, drinking and activity

Report nausea, mouth discomfort, diarrhoea or weight loss before they make eating and drinking difficult. The team can adjust symptom support and arrange dietitian or rehabilitation advice.

Skin checks, sun protection and optional photographs

This complements clinical assessment and does not replace it.

Protect your skin

Protect exposed skin with clothing, a hat and shade. Use broad-spectrum sunscreen with sun protection factor (SPF) 30 or higher and reapply as directed. Avoid sunbeds. Cloudy weather does not remove ultraviolet exposure.

Know the pattern of your skin

Ask the team to show you how to check your skin and treated area. Use a mirror or a helper for areas you cannot see. Report a new lump, a changing lesion or a sore that does not heal.

Photographs are optional

A private, dated photograph can help describe a change if your team finds it useful. Keep the lighting and size reference consistent. Photographs support a discussion; they do not replace examination and need not be uploaded.

Wounds, nails and sensitive skin

Follow the wound and skin-care instructions for the treated area. Ask before adding a cream or remedy. Increasing redness, discharge, pain, bleeding or difficulty using the area needs assessment.

Pregnancy, fertility and protecting others

Discuss these precautions before treatment even if pregnancy is not currently planned.



Medicine-specific precautions

Planning ahead

Ask early about fertility preservation, sexual health and symptoms affecting intimacy. Tell the team about any chance of pregnancy, breastfeeding or a partner's pregnancy so that the correct medicine-specific advice can be given. Keep the precautions and their end dates in the written plan.

Safe handling and supply

Do not share anticancer medicines. Keep oral medicines in their original labelled packaging, away from children and anyone who could take them accidentally. Return unused medicines to the supplying pharmacy and follow its handling advice. A treatment hold does not remove pregnancy precautions.



Testing and medicine supply

When pregnancy is possible, the initial prescription and dispensing must occur by day seven, counting the pregnancy-test date as day one. Supplies are limited to 28 days and need a renewed prescription. Ask the team to coordinate supervised monthly testing and supply so that these safeguards are maintained.

Questions worth asking

When are my next tests?

Full blood count, kidney and liver tests every four weeks. Review cramps, strength, taste, eating, weight, hair/skin changes and treatment tolerance. Pregnancy testing is monthly when applicable, even if periods stop. Your team adds the actual dates.

Can I use this without a smartphone?

Yes. The full handbook, three-page summary and wallet card are printable. The QR code is optional; keep actual contacts and written appointments on the paper copy.

Try explaining the plan back: Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

Your diagnosis and treatment setting NCCP 00236: symptomatic metastatic basal cell carcinoma, or locally advanced basal cell carcinoma unsuitable for surgery or radiotherapy.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous oral treatment

Active medicines: Vismodegib

Emetogenic risk: Minimal to low (oral)

Medicine	Dose / route	When
Individual anti-sickness / rescue plan	To be completed by the prescribing team if needed	As specified in the individual plan; no routine schedule issued here Source emetogenic category and local policy guide the decision. No medicine, dose or administration order is selected by this draft.

Additional phase-specific support

- Formal pregnancy-prevention programme, nutrition/taste and muscle symptom review. No source-mandated routine creatine phosphokinase (CPK) interval or automatic supplement prescription.
- Vismodegib can cause severe harm to an unborn baby. Women who could become pregnant need the formal pregnancy-prevention programme, including two recommended contraceptive methods (one highly effective method plus a barrier method), unless the agreed alternative is continuous abstinence, during treatment and for 24 months after stopping. Pregnancy tests are supervised before treatment and monthly, even with absent periods. Men must use a condom, with spermicide if available, during sex with a female partner during treatment and for two months afterwards, including after a vasectomy. Report a suspected patient or partner pregnancy immediately. Do not breastfeed or donate blood during treatment or for 24 months afterwards; do not donate semen during treatment or for two months afterwards. Ask the team to explain and document the full programme; this summary does not replace its materials.
- Call urgently for blistering or peeling skin, mouth sores with rash or fever, severe muscle pain or weakness, or dark urine. Seek immediate advice about suspected pregnancy or exposure. Report taste change, reduced eating and weight loss early so support can start.
- This is not an issued prescription. The clinician and pharmacist must complete and authorise any supportive medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Energy / daily activity

Vismodegib

Tiredness can affect daily tasks. Tell the team if it worsens or comes with dizziness, reduced eating or other new symptoms.

2 Hair / scalp

Vismodegib

Hair may thin or fall out. Ask about scalp care, sun protection and practical support; the timing of recovery varies.

3 Taste / eating

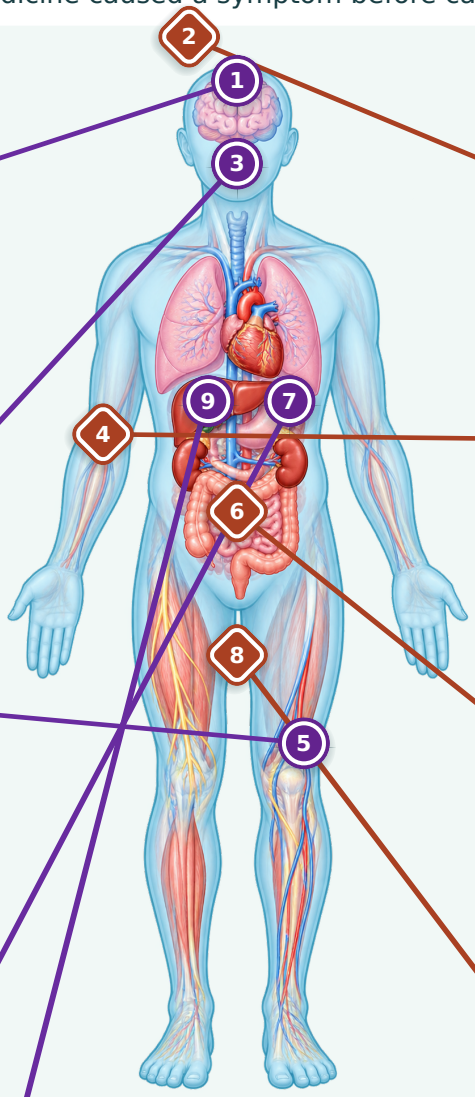
Vismodegib

Taste may change or disappear, making food less appealing. Report reduced intake early and ask for dietitian help.

4 Skin

Vismodegib

A rash with blistering, peeling, mouth sores or fever needs urgent assessment. Keep scheduled skin reviews and report new lesions.



5 Muscles / strength

Vismodegib

Cramps and muscle pain are common concerns. Severe pain, weakness or dark urine needs urgent advice; do not add supplements or medicines yourself.

6 Stomach / bowels

Vismodegib

Nausea, diarrhoea or constipation may occur. Call if symptoms persist, you cannot drink or urine becomes reduced.

7 Appetite / weight

Vismodegib

Taste changes and poor appetite can cause weight loss. Keep a practical eating plan and tell the team about loose clothes or falling weight.

8 Pregnancy / fertility

Vismodegib

A strict pregnancy-prevention programme applies to women and men, with restrictions continuing after treatment. Report possible exposure immediately.

9 Liver

Vismodegib

Report yellow skin or eyes, dark urine, unusual itching or upper-abdominal pain. Keep the planned liver blood tests; a symptom needs assessment rather than self-diagnosis.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call urgently for blistering or peeling skin, mouth sores with rash or fever, severe muscle pain or weakness, or dark urine. Seek immediate advice about suspected pregnancy or exposure. Report taste change, reduced eating and weight loss early so support can start.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00236 v3: current regimen source**
   https://healthservice.hse.ie/documents/6725/236_v3_Vismodegib.pdf
2. **EMA Erivedge product information, updated 5 November 2025 ↗**
   https://www.ema.europa.eu/en/documents/product-information/erivedge-epar-product-information_en.pdf
3. **ERIVANCE original study: Sekulic and colleagues, NEJM 2012**
   https://pubmed.ncbi.nlm.nih.gov/22670903/
4. **ERIVANCE independent-review follow-up: Sekulic and colleagues, JAAD 2015**
   https://pubmed.ncbi.nlm.nih.gov/25981002/
5. **EMA: Erivedge medicine overview**
   https://www.ema.europa.eu/en/medicines/human/EPAR/erivedge
6. **NCI: eating and nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
7. **NCI: palliative care during cancer treatment**
   https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet
8. **NCI: skin cancer — patient information**
   https://www.cancer.gov/types/skin/patient/skin-treatment-pdq
9. **NCI: skin cancer treatment — clinical evidence**
   https://www.cancer.gov/types/skin/hp/skin-treatment-pdq
10. **HSE SunSmart: protection from sun and sunbeds**
    https://www.hse.ie/conditions/cancer/protect-skin-sun-sunbeds/